Clinical Study Summary Report (CSSR) Document Status: [Draft] | **Version:** [1.0] | **Date:** [03-Apr-2026]

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1. Administrative & Study Identification

Full Study Title: A Phase 1, Open-Label, Dose Escalation and Dose Expansion Study to Evaluate the Safety, Tolerability, Pharmacokinetics, and Anti-Tumor Activity of PF-07799933 (ARRY-440) as a Single Agent and in Combination Therapy in Participants 16 Years and Older With Advanced Solid Tumors With BRAF Alterations. **Short Title/Acronym:** PF-07799933 (Claturafenib) in People With Advanced Solid Tumors With BRAF Alterations. **Protocol Number:** C4761001 **NCT Number:** NCT05355701 **Trial Status:** RECRUITING **Sponsor Name:** Pfizer **Investigational Product:** PF-07799933 (Claturafenib), alone or in combination with Binimetinib, Cetuximab, or mFOLFOX6. **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Clinical Trial Contact Center

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and pharmacokinetics of PF-07799933, and to determine the Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D). **Secondary Objectives:** To evaluate the anti-tumor activity (Objective Response Rate [ORR], Intracranial Response, and Duration of response) of PF-07799933 as a single agent and in combination therapy.

2.2 Background & Rationale Alterations in the BRAF gene can lead to the abnormal growth and spread of cancer cells. While RAF inhibitors have transformed treatment for patients with BRAF V600-mutant cancers, clinical benefit is often limited by acquired resistance and poor brain penetration. Next-generation pan-RAF dimer inhibitors have been limited by a narrow therapeutic index. This study evaluates PF-07799933, a novel brain-penetrant, selective, pan-mutant BRAF inhibitor, designed to overcome resistance mechanisms in patients whose advanced solid tumors no longer respond to available treatments.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose escalation and dose expansion cohorts) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 156 **Number of Sites:** 40 locations globally (including US, Canada, Israel) **Estimated Study Start:** 05-Jul-2022 **Estimated Primary Completion:** 02-Nov-2028 (Estimated End Date)

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 16 years. 2. Confirmed diagnosis of an advanced/metastatic solid tumor including primary brain tumor. 3. Documented qualifying BRAF alteration (V600 or non-V600 Class II/Class III BRAF alteration), in tumor tissue and/or blood (ctDNA). 4. Disease progressed during/following last prior treatment and no satisfactory alternative treatment options (Part 1 and Part 2). 5. Tumor-specific cohorts (melanoma, colorectal cancer) must have received specific prior approved therapies (e.g., prior BRAF/MEK/checkpoint inhibitor for melanoma; 5-FU/checkpoint inhibitor for CRC).

4.2 Exclusion Criteria 1. Brain metastasis larger than 4 cm. 2. Systemic anti-cancer therapy or small molecule therapeutics ongoing at the start of study treatment. 3. For participants who may get binimetinib on study: history or current evidence of retinal vein occlusion (RVO) or concurrent neuromuscular disorder associated with elevated creatine kinase (CK).

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **PF-07799933:** Administered as a tablet taken by mouth, twice daily. * **Binimetinib (Combination Arm):** Administered as a tablet taken by mouth, twice daily. * **Cetuximab (Combination Arm for CRC):** Administered weekly (or every two weeks) via intravenous (IV) infusion. **Route of Administration:** Oral (PF-07799933, Binimetinib), IV (Cetuximab, mFOLFOX6). **Duration of Treatment:** Expected to last around 2 years, or until disease progression/ unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care. **Prohibited:** Concurrent systemic anti-cancer therapies or small molecule therapeutics at the start of the study.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Prior to Cycle 1	Informed Consent, medical history, baseline imaging, tissue/blood BRAF mutation confirmation, physical exam
Cycle 1	Day 1 to 21	First Dose, intense Pharmacokinetic (PK) sampling, primary Dose-Limiting Toxicity (DLT) observation period
Treatment Cycles	Every 21 Days	Continuous safety labs, physical examinations, heart function tests, and continuous study drug dispensing
Efficacy Assessments	Per Protocol	Tumor assessments (imaging scans) measured via RECIST v1.1 / RANO

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definitions: 1. **Number of participants with dose limiting toxicities (DLTs)** (Part 1 and Part 2) evaluated during the first cycle (21 days) as both a single agent or in combination with binimetinib or cetuximab. 2. **Number of participants with treatment-emergent adverse events (AEs)** (Part 1 and Part 2) as characterized by type, frequency, severity, timing, seriousness, and relationship to study therapy. 3. **Number of participants with clinically significant change from baseline in laboratory abnormalities** (Part 1 and Part 2) as characterized by type, frequency, severity, and timing.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR) (Part 1 and Part 2) assessed using RECIST version 1.1 for solid tumors. * Intracranial response (Part 1/2/3) assessed by Response Assessment in Neuro-Oncology (RANO) for primary brain tumors, and RECIST v1.1 for brain metastases. * Duration of response (Part 1 and Part 2). * Pharmacokinetics of PF-07799933.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of adverse events (AEs) and serious adverse events (SAEs) through clinical visits, health questionnaires, blood/urine samples, and cardiac function testing. **Serious Adverse Events (SAEs):** Routine Phase 1 pharmacovigilance timeline (typically 24 hours). **Dose-Limiting Toxicities (DLTs):** Specifically monitored during the 21-day Cycle 1 to inform the pharmacokinetics-informed dose escalation model.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all participants who receive at least one dose of study drug). **Sample Size Justification:** Targeted enrollment of 156 participants is standard for Phase 1 dose-escalation/expansion; driven by the number of cohorts and observed dose-limiting toxicities. **Handling of Missing Data:** Appropriate imputation methods as defined by the study's SAP for an open-label, non-randomized Phase 1 trial. Dose escalation utilizes a novel, flexible, pharmacokinetics-informed design.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Continuous site monitoring to ensure protocol adherence, drug accountability, and timely reporting of DLTs. **Audit Trail:** eCRF locking and query resolution via standard clinical data management systems.

11. Study Identifiers & Governance

Primary ID: C4761001 **Registry Number:** NCT05355701 **Posting ID:** 684924083705972343 **Sponsor/Lead Organization:** Pfizer **Company:** Pfizer **Study Title:** A Study to Learn About the Study Medicine Called PF-07799933 in People With Advanced Solid Tumors With BRAF Alterations. **Official Regulatory Title:** A PHASE 1, OPEN-LABEL, DOSE ESCALATION AND DOSE EXPANSION STUDY TO EVALUATE THE SAFETY, TOLERABILITY, PHARMACOKINETICS, AND ANTI TUMOR ACTIVITY OF PF-07799933 (ARRY-440) AS A SINGLE AGENT AND IN COMBINATION THERAPY IN PARTICIPANTS 16 YEARS AND OLDER WITH ADVANCED SOLID TUMORS WITH BRAF ALTERATIONS.

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12. Clinical Framework (BRAF Alteration Specific)

Disease Areas: Melanoma, Non-Small-Cell Lung Cancer, Thyroid Cancer, Glioma. **Preferred UMLS Name:** BRAF Positive Neoplasms **Diagnosis Threshold:** Documented qualifying BRAF alteration (V600 or non-V600 Class II/Class III) via tumor tissue or ctDNA. **Medical Methodology:** Next-generation targeted pan-mutant BRAF inhibitor therapy. **Optimization Target:** Overcoming BRAF treatment resistance, disrupting endogenous mutant-BRAF dimers, and penetrating the blood-brain barrier.

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13. Study Procedures & Methodology

Management Pathway: Screening for advanced solid tumors followed by molecular subtyping for BRAF alterations; assignment to monotherapy or combination therapy cohorts (Binimetinib or Cetuximab based on tumor type). **Education Protocol (HCP):** Site initiation visits and regular protocol safety training regarding DLT tracking and unique PK-informed dose escalation models. **Education Protocol (Patient):** Thorough informed consent process detailing the absence of a placebo, daily oral tablet adherence, and rigorous safety screening schedules. **Quality Audit Mechanism:** Continuous safety data review to determine dose escalations and expansions.

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14. Observation & Follow-Up Schedule

Total Duration: Approximately 2 years per patient. **Onsite Visit Cadence:** Pre-defined schedule aligning with 21-day cycles. **Remote Monitoring Frequency:** Continuous remote data review by the sponsor. **Checklist Review Interval:** Safety and PK assessment check-ins heavily weighted in Cycle 1, spacing out in subsequent cycles.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]